

BACKGROUND

- Liver transplantation involves uniquely complex donor-recipient, longitudinal and multimodal clinical decision making
 - Foundation Models (FMs) such as GPT are rapidly transforming healthcare
 - Liver transplantation, however, remains underexplored with most models focused on isolated tasks
- There is a need to move beyond narrow task specific models towards domain-specific liver transplant foundation models**

DATASET CURATION AND PREPROCESSING

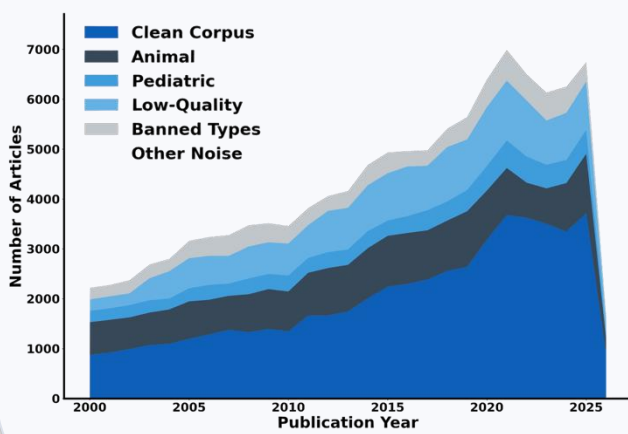


Fig 1. Data Preprocessing

- Preprocessing Summary**
- Q1/Q2 quality filtering
 - MeSH filtering
 - Structural extraction
 - Deduplication

MODEL DEVELOPMENT

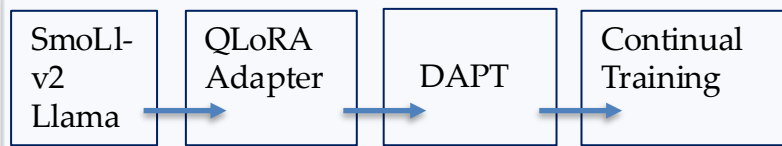


Fig 4. Modeling Pipeline

Specialized Liver Model

- QLoRA-based parameter efficient adaptation
- One training epoch
- Sequence packing and diagonal masking
- Casual Language Modeling

LIVER TRANSPLANT SPECIFIC QA

Question	Base Model	Our Model
Tacrolimus ID	FK-508	FK-506
MELD	Original MELD	MELD 3.0
HCC Eligibility	Missed Milan Criteria	Milan Criteria

The specialized model produced liver responses:

- More up-to date
- Accurate and clinically grounded
- Fewer hallucination

Table 1. Qualitative Comparison

EVALUATION AND RESULTS

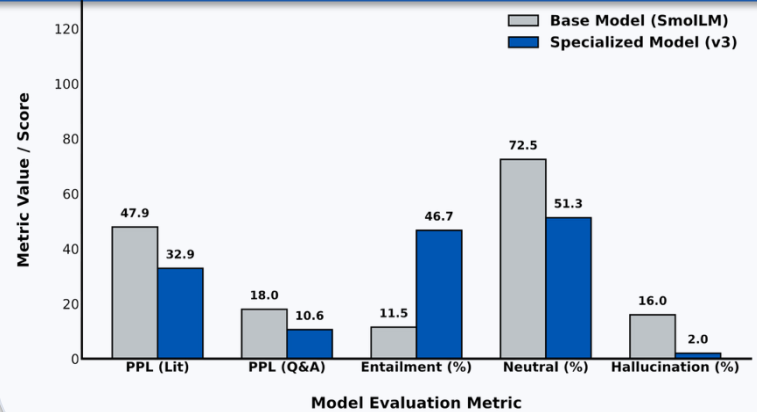


Fig 3. Quantitative Comparison

We developed a liver-transplant focused clinical QA benchmark using:

- OPTN, AASLD, textbooks, Cleveland clinic resources

Specialized Model showed:

- Reduced perplexity
- Reduced hallucinations
- Increased factual entailment

CONCLUSION

- We present the first liver-transplant foundation model
- This work lays the foundation for next-generation FMs that are clinically useful and trustworthy

FUTURE WORK

- Incorporate De-identified clinical notes
- Scale to larger Llama Models
- Incorporate RAG for protocols
- Compare to larger models (GPT)